

Self-scheduling splits into four features and nine stories

EPIC E-101

Self-scheduling

Patients book, move and cancel visits in the app without a call.

33

points, four features

31

acceptance criteria

	USER STORY	AC	POINTS	STATUS
FIND	As a patient, I want to search open slots by provider	4	3	Done
	As a patient, I want to filter by location and time	3	3	Done
BOOK	As a patient, I want to pick a visit type and book	5	8	In progress
	As a patient, I want a booking confirmation	3	2	In progress
CHANGE	As a patient, I want to reschedule from the app	4	5	In progress
	As a patient, I want to cancel and free the slot	3	3	Not started
NOTIFY	As a patient, I want a text reminder with a link	4	5	Not started
	As a patient, I want to add the visit to my calendar	2	2	Not started
	As a patient, I want a day-before prep note	3	2	Blocked

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

Eight epics carry the front door; access and estimates lead

	EPIC	OUTCOME	PRIORITY	SIZE	STATUS	RELEASE
ACCESS	Self-scheduling	Book and move visits without a call	P1	XL	In progress	4.1 · Nov
	Waitlist and cancellations	Refill released slots within hours	P2	L	In progress	4.2 · Jan
	Digital intake	Arrive with forms and consents done	P1	L	In progress	4.1 · Nov
MONEY	Cost estimates	Show the patient price before the visit	P1	XL	Not started	4.2 · Jan
	Bill pay	Pay a statement in two taps	P2	M	Done	4.0 · Oct
	Payment plans	Set up a plan without calling billing	P3	L	Not started	4.3 · Mar
ENGAGE	Secure messaging	Reach the care team in one thread	P2	L	Blocked	4.2 · Jan
	Proxy access	Let caregivers act for family members	P4	M	Not started	4.3 · Mar

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

The next two sprints are ready; the tail is aging

42

ready items, of 118 open

88%

refined for the next two sprints

47

days average backlog age

■ AT RISK

6

blocked items

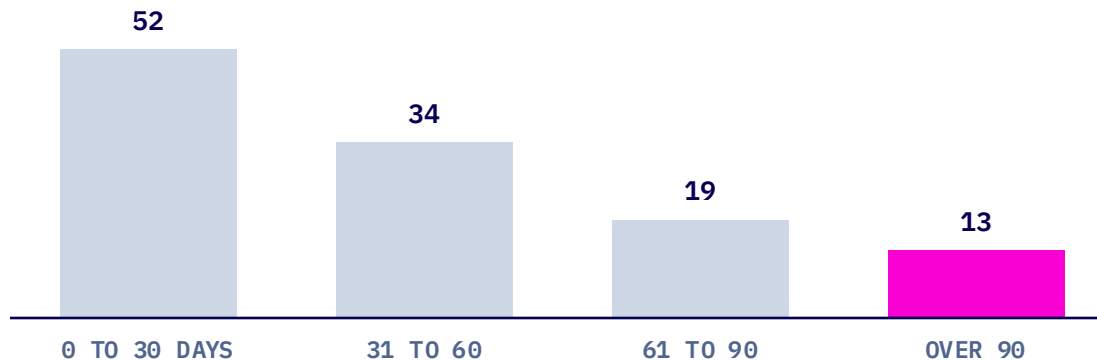
■ AT RISK

28%

of open items are bugs,
72% features

Open items by age

ITEMS



- 01 Refinement is on track**
Next two sprints hold 88% refined stories.
- 02 Messaging is stuck**
Five of six blocked items wait on the vendor API.
- 03 Old items need a verdict**
13 items over 90 days: close, split or schedule.

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

A story must be ready to start and done to count

DEFINITION OF READY

A story enters a sprint when

- ✓ Story states who, what and why
- ✓ Acceptance criteria are testable
- ✓ Designs are linked and reviewed
- ✓ Dependencies are named and clear
- ✓ Estimated by the team at 8 points or less
- ✓ Product owner has ranked it
- ✓ Fits in one sprint

DEFINITION OF DONE

A story is done when

- ✓ Code reviewed and merged
- ✓ Unit and UI tests pass
- ✓ Accessibility check passed
- ✓ Analytics events verified
- ✓ Release notes drafted
- ✓ Product owner accepted it
- ✓ Support team briefed

The product owner owns ready, the delivery team owns done, and both are reviewed in the quarterly retro.

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

RELEASE PLAN

Three releases take the front door through March

RELEASE 4.1 • NOV 2026

01 Book and arrive ready

- Self-scheduling · 33 pts
- Digital intake · 20 pts

OUTCOME

Patients book, move and arrive with forms done.

RELEASE 4.2 • JAN 2027

02 Know the price first

- Cost estimates · 34 pts
- Waitlist offers · 18 pts
- Messaging · 8 pts

OUTCOME

Patients see the price before they commit.

RELEASE 4.3 • MAR 2027

03 Pay and share care

- Payment plans · 21 pts
- Proxy access · 13 pts
- Hardening · 14 pts

OUTCOME

Patients settle bills and share care with family.

53 of 60

points planned against capacity

60 of 62

points planned against capacity

48 of 60

points planned against capacity

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

RANKED BACKLOG

Twelve ranked items; the top five fill two sprints

- **Value** patient time saved and calls avoided.
- **Urgency** payer and vendor dates that cannot move.
- **Effort** team sizing, in S to XL tags.
- **Risk** unblock the messaging vendor first.

CUT LINE

Top 5

fit 48 points, two sprints at capacity.

#	BACKLOG ITEM	EPIC	PRIORITY	SIZE	SPRINT
1	Saved cards for bill pay	Bill pay	P1	S	S25
2	Reschedule from reminder	Self-scheduling	P1	M	S25
3	Open-slot filters	Self-scheduling	P2	M	S26
4	Estimate at booking	Cost estimates	P2	XL	S26
5	Intake pre-fill	Digital intake	P2	L	S26
6	Waitlist offers	Waitlist	P2	L	S27
7	Message threads	Messaging	P2	L	S27
8	Payment plan setup	Payment plans	P3	L	S27
9	Proxy invite reminders	Proxy access	P3	S	Later
10	Message attachments	Messaging	P3	M	Later
11	Family calendar view	Proxy access	P4	M	Later
12	Wayfinding maps	Wayfinding	P4	XL	Later

Sprints 24 to 27 commit to 24 points each

Sprint 24

OCT 12 TO 23 • CAPACITY 28

Saved cards · 4
Reschedule link · 6
Cancel, free slot · 4
Open-slot filters · 6
Confirmation · 4

Sprint 25

OCT 26 TO NOV 6 • CAPACITY 28

Estimate API · 8
Intake pre-fill · 8
Calendar add · 2
Waitlist offers · 6

Sprint 26

NOV 9 TO 20 • CAPACITY 28

Estimate screen · 8
Message threads · 6
Prep note · 4
Bug burn-down · 6

Sprint 27

NOV 30 TO DEC 11 • CAPACITY 28

Payment plan setup · 8
Proxy invites · 4
Attachments · 4
Hardening · 6

COMMITMENT 24 PTS

 Carryover from Sprint 23

 Planned story, points shown after the name

Reschedule from a text reminder in under a minute

USER STORY

As a patient, I want to move a visit from the reminder text, so that I keep my care without calling.

ACCEPTANCE CRITERIA

GIVEN	WHEN	THEN
A reminder text 48 hours out	I tap the link and sign in	My visit opens with a Reschedule action
Open slots with the same provider	I choose a new slot	The old slot is freed and I get a confirmation
No open slots in 14 days	I tap Reschedule	I am offered the waitlist or a call-back

Designs: Figma link placeholder, reschedule flow

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

5 pts

estimate, planned for Sprint 25

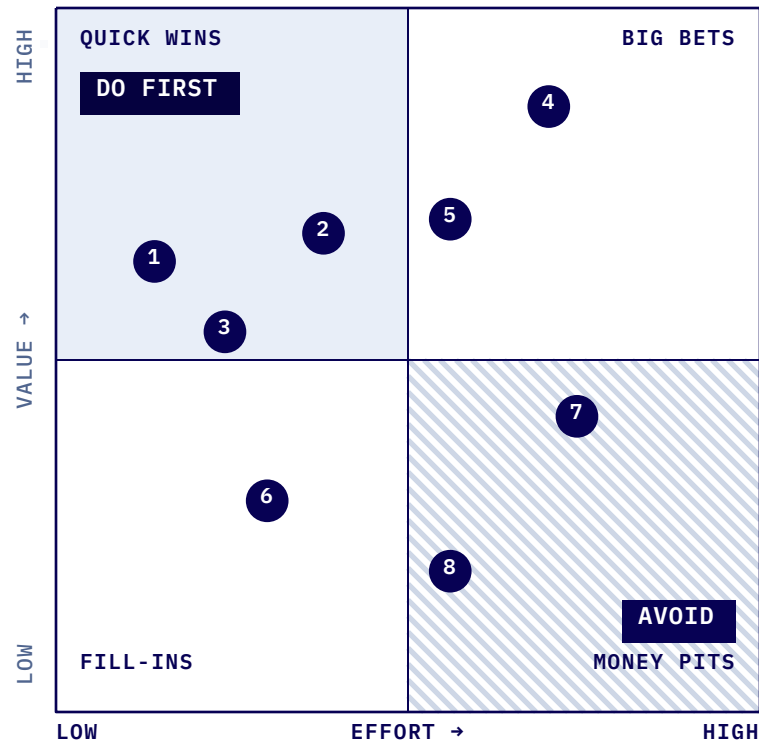
DONE	DEFINITION OF DONE
☒	Code reviewed and merged
☒	Unit and UI tests pass
☐	Accessibility check passed
☐	Product owner accepted

DEPENDS ON

- NFHS-198 slot search API
- Text reminder deep links

Three quick wins come first; wayfinding waits

Value and effort



RANKED BY VALUE OVER EFFORT

READING THE 2X2

1. Reschedule from reminder
2. Open-slot filters
3. Saved cards for bill pay
4. Estimate at booking
5. Intake pre-fill
6. Proxy invite reminders
7. Family calendar view
8. Wayfinding maps

- 01 Quick wins**
Items 1 to 3 go into the next sprint.
- 02 Big bets**
Items 4 and 5 are sliced before commitment.
- 03 Hold**
Items 6 to 8 wait for capacity.

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

Weighted scoring puts bill pay and reschedule at the top

BACKLOG ITEM	VALUE	TIME CRITICALITY	RISK REDUCTION	JOB SIZE	WSJF SCORE	RANK	TIER
Saved cards for bill pay	5	5	3	2	6.5	1	P1
Reschedule from reminder	8	8	3	3	6.3	2	P1
Open-slot filters	5	3	2	3	3.3	3	P2
Estimate at booking	13	13	8	13	2.6	4	P2
Intake pre-fill	8	5	5	8	2.2	5	P2
Proxy invite reminders	3	2	3	5	1.6	6	P3
Message attachments	3	2	2	5	1.4	7	P3
Wayfinding maps	2	1	1	8	0.5	8	P4

WSJF = (value + time criticality + risk reduction) divided by job size, each on a 1, 2, 3, 5, 8, 13 scale.

Source: Northfield digital front door backlog, Oct 2026. Sample figures for layout only.

Two roles are short at peak and one is over-resourced

ROLE	PEAK DEMAND (FTE)	PLANNED CAPACITY (FTE)	VARIANCE	STATUS
Engagement lead	0.5	0.5	—	■ Covered
Product manager	1	1	—	■ Covered
Solution architect	1	1	—	■ Covered
Data engineer	1	0.5	▼ -0.5	■ Gap
Business analysts	2	2	—	■ Covered
Developers	3	2	▼ -1.0	■ Gap
QA lead	1	1	—	■ Covered
Change lead	1	1.5	▲ +0.5	■ Over

Net peak position: 1.5 FTE short on developers and data engineering, 0.5 FTE over on change.

RESOURCE PLAN

Each firm role has a
named client
counterpart

- Counterparts hold decision rights for their area.
- Hours are per week at peak and fall in launch.
- A gap in any counterpart slows the matching firm role.

62 hrs

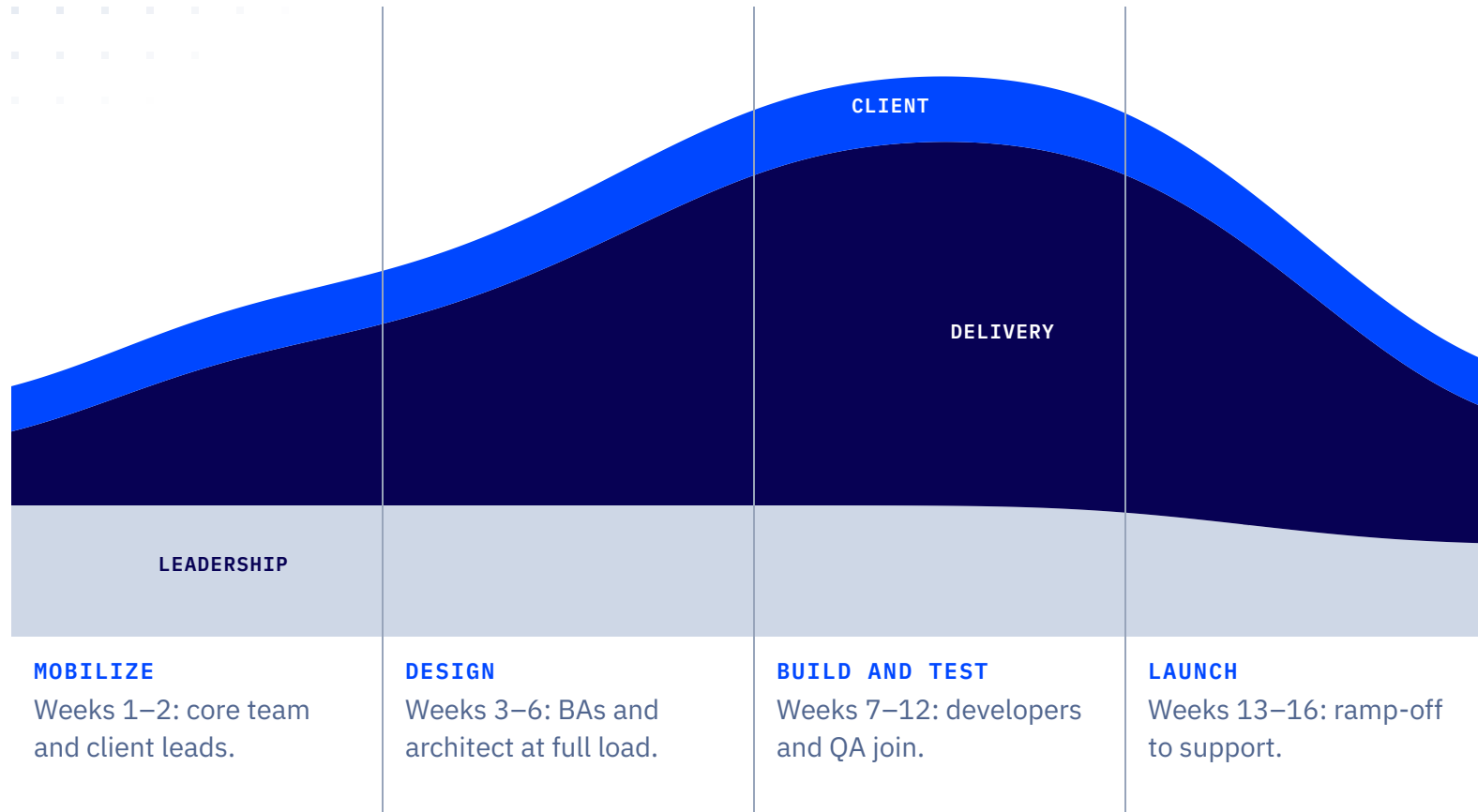
client time per week at peak

7

named counterparts

FIRM ROLE	CLIENT COUNTERPART	HRS PER WEEK
Engagement lead	Executive sponsor	2
Product manager	Product owner	20
Solution architect	IT integration lead	10
Data engineer	Data steward	6
Business analysts	Patient access SMEs	8
QA lead	UAT coordinator	10
Change lead	Department managers	6
Client commitment	Seven counterparts	62

The team peaks at 10.8 FTE, then rolls off



10.8 FTE

peak team size, weeks 7-10

7.9 FTE

average over 16 weeks

4.6 FTE

in week 16, handing over to run

1.2 FTE

client team at peak

A firm team of up to 9.5 FTE delivers in 16 weeks

ROLE (FIRM FTE)	MONTH 1	MONTH 2	MONTH 3	MONTH 4
Engagement lead	0.5	0.5	0.5	0.5
Product manager	1	1	1	0.5
Solution architect	1	1	0.5	0.25
Data engineer	0.5	1	1	0.5
Business analysts (2)	2	2	1	1
Developers (3)	–	3	3	1
QA lead	–	0.5	1	1
Change lead	0.5	0.5	1	1
Total firm FTE	5.5	9.5	9	5.75

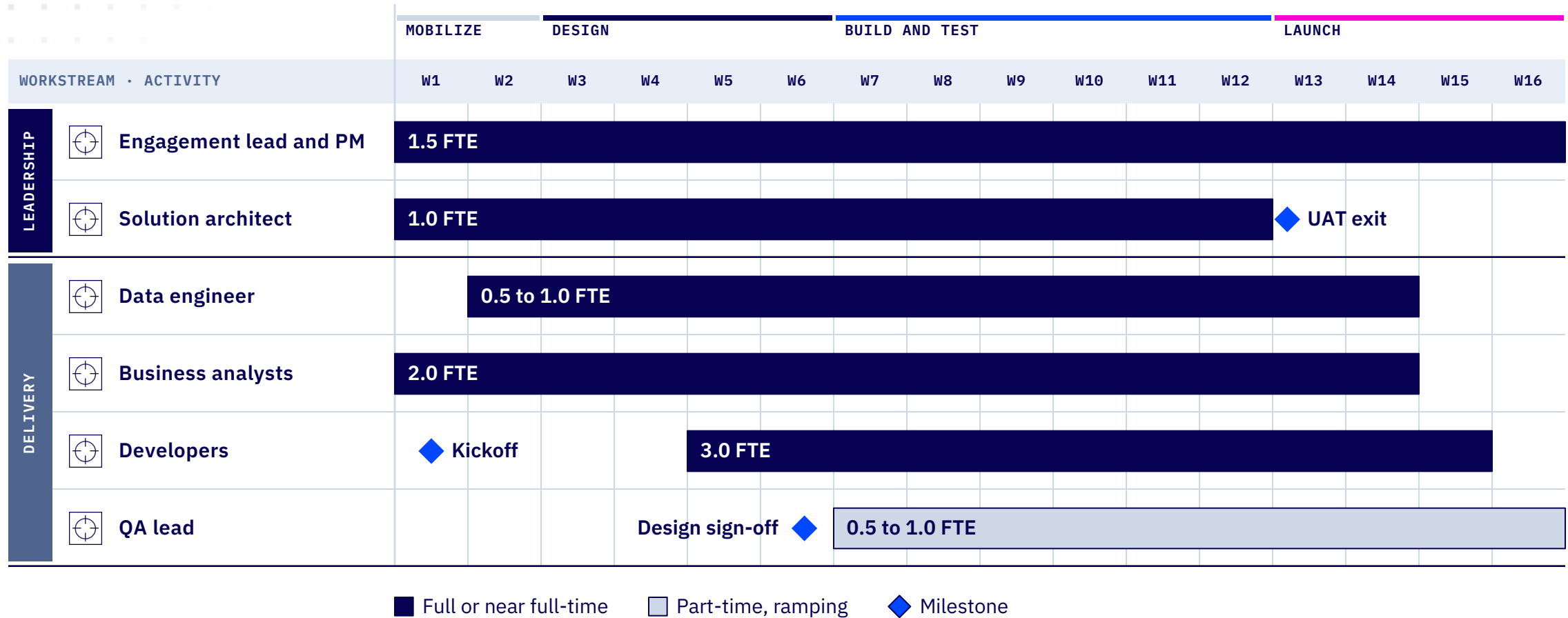
KEY ASSUMPTIONS

- Client leads are named by week 1.
- Developers start in week 5.
- Two-thirds of build is onshore.

PHASE	TEAM RUN RATE / MO
Mobilize	\$190K
Design	\$310K
Build and test	\$460K
Launch	\$330K
Peak monthly run rate	\$460K

Source: Sample figures for layout only; replace with current firm data before use.

Roles join as work needs them and roll off early



Nine named people are in place by February 16

MR **Mara R.**
Engagement lead

50% • JAN 5 – APR 24

DK **Devon K.**
Product manager

100% • JAN 5 – APR 17

SP **Sasha P.**
Solution architect

100% TO 25% • JAN 5 – MAR 27

TL **Tariq L.**
Data engineer

50% TO 100% • JAN 12 – APR 10

AN **Anya N.**
Business analyst

100% • JAN 5 – APR 10

JO **Jonas O.**
Business analyst

100% • JAN 5 – APR 10

PC **Priya C.**
Lead developer

100% • FEB 2 – APR 24

RB **Rowan B.**
QA lead

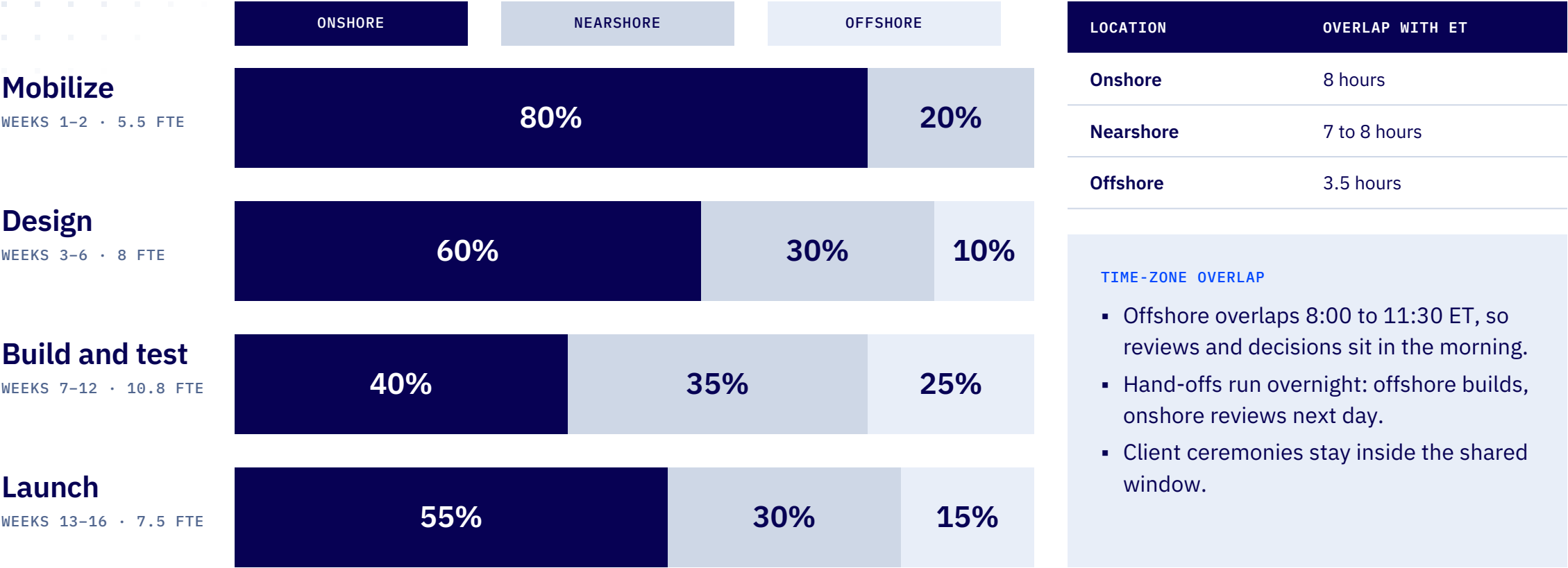
50% TO 100% • FEB 16 – APR 24

EH **Elena H.**
Change lead

50% TO 100% • JAN 19 – APR 24

Allocation is a share of a 40-hour week; ranges show ramp-up or ramp-down.

Offshore share grows as the build scales up



Source: Sample figures for layout only; replace with current firm data before use.

Loading peaks in weeks 7–8 and eases through launch

	ROLE	W1–2	W3–4	W5–6	W7–8	W9–10	W11–12	W13–14	W15–16	AVG FTE
LEADERSHIP	Engagement lead	0.5	0.5	0.5	0.5	0.5	0.5	0.5	0.5	0.5
	Product manager	1	1	1	1	1	1	1	0.5	0.94
	Solution architect	1	1	1	1	0.5	0.5	0.25	0.25	0.69
DELIVERY	Data engineer	0.5	0.5	1	1	1	1	0.5	0.5	0.75
	Business analysts (2)	2	2	2	2	1	1	1	1	1.5
	Developers (3)	–	–	2	3	3	3	2	1	1.75
	QA lead	–	–	0.5	0.5	1	1	1	1	0.62
	Change lead	0.5	0.5	0.5	0.5	1	1	1	1	0.75
CLIENT	Executive sponsor	0.1	0.1	0.1	0.1	0.1	0.1	0.1	0.1	0.1
	Product owner	0.5	0.5	0.5	0.5	0.5	0.5	0.5	0.5	0.5
	IT integration lead	0.25	0.25	0.5	0.5	0.5	0.5	0.25	0.25	0.38
	Total FTE	6.3	6.3	9.6	10.6	10.1	10.1	8.1	6.6	8.47

Peak fortnight is weeks 7–8 at 10.6 FTE; client counterparts hold near 1 FTE throughout.

Staffing peaks at 10.6 FTE in month 2

	ROLE	MONTH 1	MONTH 2	MONTH 3	MONTH 4	AVG FTE
LEADERSHIP	Engagement lead	0.5	0.5	0.5	0.5	0.5
	Product manager	1.0	1.0	1.0	0.5	0.9
	Solution architect	1.0	1.0	0.5	0.25	0.7
DELIVERY	Data engineer	0.5	1.0	1.0	0.5	0.8
	Business analysts (2)	2.0	2.0	1.0	1.0	1.5
	Developers (3)	–	3.0	3.0	1.0	1.8
	QA lead	–	0.5	1.0	1.0	0.6
	Change lead	0.5	0.5	1.0	1.0	0.8
CLIENT	Executive sponsor	0.1	0.1	0.1	0.1	0.1
	Product owner	0.5	0.5	0.5	0.5	0.5
	IT integration lead	0.25	0.5	0.5	0.25	0.4
	Total FTE	6.3	10.6	10.1	6.6	8.4

Peak is month 2: 10.6 FTE, of which the client team provides 1.1.

Every bet names its assumption and its decision date

HORIZON	STRATEGIC BET	ASSUMPTION TO VALIDATE	SIGNAL OF SUCCESS	DECISION POINT
NOW	Master patient index	Matching rules reach 6% duplicates without manual review at every site.	Duplicates under 8% at two hospitals by Q1 27.	Decide Mar 2027 Buy a matching service if above 9%.
NOW	Single sign-in	Patients and staff accept one identity without a second login.	70% of active patients enrolled within 90 days.	Decide Jun 2027 Fall back to staff-assisted enrolment.
NEXT	Self-scheduling	Patients book online when slots are real time and complete.	30% of primary care visits booked online.	Decide Sep 2027 Pivot to staff-assisted booking if under 20%.
NEXT	Digital intake	Patients finish forms and insurance before they arrive.	60% complete intake before the visit.	Decide Dec 2027 Cut to insurance capture only if under 40%.
LATER	Price estimates	Estimates within 10% of the final bill build trust.	Front-end denials under 6%.	Decide Jun 2028 Narrow to scheduled procedures if misses persist.

Source: Sample figures for layout only; replace with current firm data before use.

Three bets carry the vision into a sequenced roadmap

PRODUCT VISION

Every Northfield patient reaches, books and pays for care in one digital front door.

1 · Know every patient

OUTCOME

Duplicate records 11% → 6%

One record, one sign-in everywhere.

Now · Master patient index

Next · Single sign-in

Later · Shared patient data store

2 · Open the door

OUTCOME

Visits booked online 8% → 60%

Patients book and arrive ready, no call.

Now · Self-scheduling, primary care

Next · Digital intake

Later · Multilingual booking

3 · Pay and track with confidence

OUTCOME

Front-end denials 9% → 5%

Price and eligibility known up front.

Now · Eligibility checks

Next · Price estimates

Later · Referral tracking

Source: Sample figures for layout only; replace with current firm data before use.

Every initiative traces back to the vision

PRODUCT VISION

Every Northfield patient reaches, books and pays for care in one digital front door.

NORTH STAR

80% of patients complete access online by 2028.

Baselines are 2026 averages across six hospitals.

Source: Sample figures for layout only; replace with current firm data before use.

	OUTCOME	BASELINE → TARGET	INITIATIVES	HORIZON
KNOW THE PATIENT	Duplicate patient records	11% → 6%	Master patient index	NOW
	Sites on one sign-in	0 → 6 of 6	Single sign-in	NOW
OPEN THE DOOR	Visits booked online	8% → 60%	Self-scheduling; digital intake	NEXT
	Calls to the access center	100% → 75%	Waitlist; reminders	NEXT
PAY AND TRACK	Front-end denials	9% → 5%	Eligibility checks; price estimates	LATER
	Referrals completed	61% → 85%	Referral tracking	LATER

Strategy to roadmap: pillars, bets and decision gates

STRATEGIC PILLAR

Now

Next

Later

OUTCOME • 11% → 6% DUPLICATES

Know every patient

Owner: chief data officer

DECISION • MAR 27

Master patient index

Signal: duplicates under 8%

DECISION • JUN 27

Single sign-in

Signal: 70% enrolled in 90 days

GATE • 2028

Shared data store

Waits for index and sign-in

OUTCOME • 8% → 60% BOOKED ONLINE

Open the door

Owner: patient access officer

DECISION • SEP 27

Self-scheduling

Signal: 30% of visits online

DECISION • DEC 27

Digital intake

Signal: 60% intake before visit

GATE • 2028

Multilingual booking

Booking in every language

OUTCOME • 9% → 5% DENIALS

Pay and track

Owner: VP revenue cycle

GATE • Q2 27

Eligibility checks

Signal: denials under 7%

DECISION • JUN 28

Price estimates

Signal: estimates within 10%

GATE • 2028

Referral tracking

One status view, order to visit

Source: Sample figures for layout only; replace with current firm data before use.

One north star, four inputs, the bets that move them

North star · patients who complete access online, 24% → 80% by 2028

Identity input

DUPLICATE RECORDS

11% → 6%

BETS THAT MOVE IT

NOW · Q4 26

Master patient index

NEXT · Q1 27

Single sign-in

Booking input

VISITS BOOKED ONLINE

8% → 60%

BETS THAT MOVE IT

NOW · Q1 27

Self-scheduling

LATER · 2028

Multilingual booking

Readiness input

INTAKE DONE BEFORE ARRIVAL

15% → 70%

BETS THAT MOVE IT

NEXT · Q2 27

Digital intake

NEXT · Q3 27

Smart reminders

Price clarity input

FRONT-END DENIALS

9% → 5%

BETS THAT MOVE IT

NEXT · Q2 27

Eligibility checks

LATER · Q3 27

Price estimates

Source: Sample figures for layout only; replace with current firm data before use.

Key results by quarter, each tied to its initiative

	OBJECTIVE	KEY RESULT	INITIATIVE MAPPED	OWNER	CONFIDENCE
Q4 26	Know every patient once	Duplicate records 11% → 9%	Master patient index	Chief data officer	High
		Sites on one sign-in 0 → 2 of 6	Single sign-in, pilot sites	Chief digital officer	Medium
Q1 27	Book without a call	Visits booked online 8% → 20%	Self-scheduling, primary care	Chief patient access officer	High
		Sites on one sign-in 2 → 4 of 6	Single sign-in, rollout	Chief digital officer	Medium
Q2 27	Arrive ready for the visit	Intake done before arrival 15% → 40%	Digital intake	Director of registration	Medium
		No-show rate 11% → 9%	Smart reminders	Chief patient access officer	Low

Source: Sample figures for layout only; replace with current firm data before use.

Every roadmap item is an outcome with a target

Now · Q4 26 to Q1 27

KPI · 11% → 6% DUPLICATE RECORDS

One record per patient

- Master patient index
- Match rules at six sites
- Duplicate review queue

KPI · 0 → 6 OF 6 SITES ON ONE SIGN-IN

One identity everywhere

- Single sign-in
- Staff identity federation
- Patient app login

Next · Q2 to Q3 27

KPI · 8% → 60% VISITS BOOKED ONLINE

Patients book without a call

- Self-scheduling, primary care
- Digital intake before arrival
- Waitlist and cancellations

KPI · 11% → 6% NO-SHOW RATE

Fewer visits lost to forgetting

- Smart reminders
- Care plans in the app
- Reminder channel choice

Later · Q4 27 and 2028

KPI · 9% → 5% FRONT-END DENIALS

Price known before booking

- Eligibility checks
- Price estimates
- Payment plans

KPI · 61% → 85% REFERRALS COMPLETED

Referrals followed to the visit

- Referral tracking
- Multilingual booking
- Caregiver proxy access

Source: Sample figures for layout only; replace with current firm data before use.

Strategy on a page: the digital front door to 2028

Every Northfield patient reaches, books and pays for care in one digital front door.

TARGET SEGMENTS

- Primary care patients booking routine visits
- Specialty patients arriving by referral
- Caregivers managing care for family
- Patients with chronic conditions

PROBLEMS WE SOLVE

- No booking without a phone call
- Six hospitals, six patient identities
- Prices unknown until the bill arrives
- Referrals lost between sites

VALUE PROPOSITION

FOR PATIENTS

One sign-in to find, book, prepare for and pay for care, with the price known first.

For staff: one verified record at every site.

STRATEGIC BETS

1. Know every patient once
2. Open the door online
3. Pay and track with confidence
4. Run on one data foundation

WHAT WE WILL NOT DO

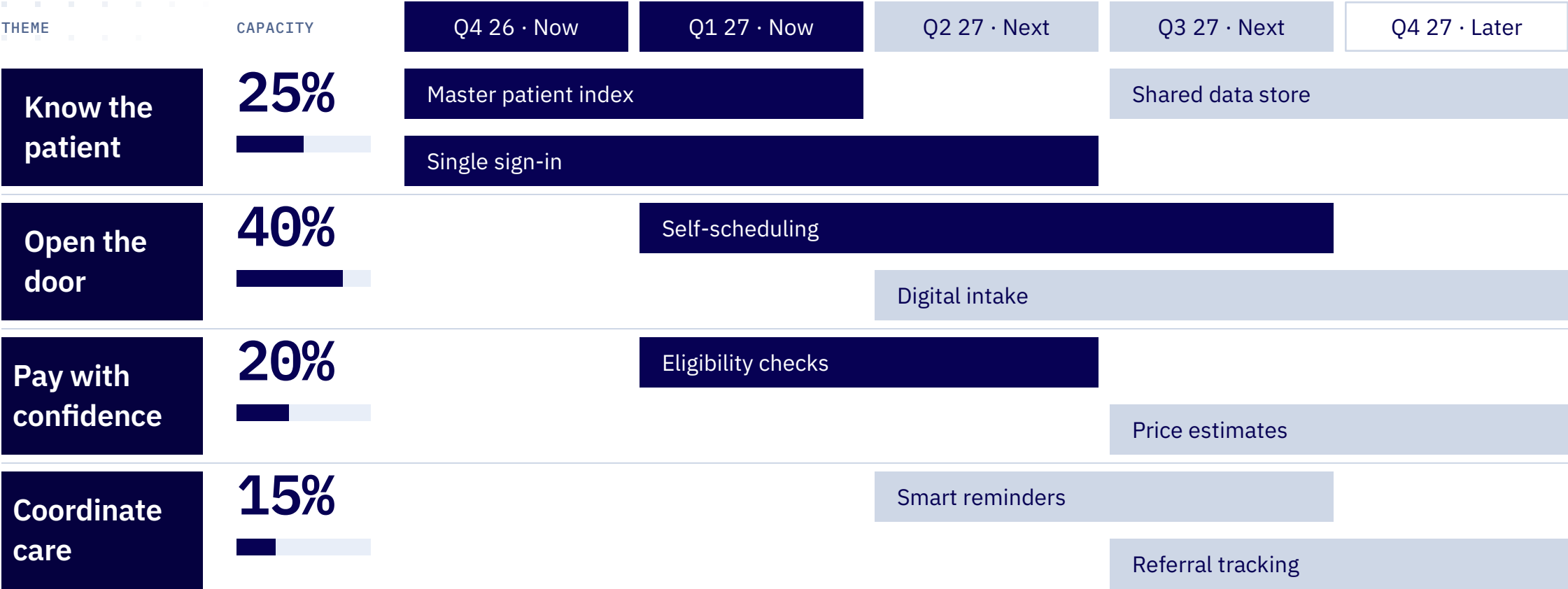
- Replace the EHR
- Build tools for a single hospital
- Add voice or kiosk channels before 2028
- Price every service before 2027

SUCCESS MEASURES

- 80% complete access online by 2028
- Calls to the access center down 25%
- Front-end denials 9% → 5%
- No-show rate 11% → 6%

Source: Sample figures for layout only; replace with current firm data before use.

Four themes share capacity across five quarters



Source: Sample figures for layout only; replace with current firm data before use.